

IFS AND HEALTH COACHING

A NEW MODEL OF BEHAVIOR CHANGE

AND MEDICAL DECISION MAKING

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INTRODUCTION

Illness often calls for the assimilation of complicated medical information and requires making tough medical decisions and difficult changes in habits of self-care. This, in turn, can activate fear, anxiety and confusion in patients and equally strong reactions in family members and health care professionals, putting patients on an emotional roller coaster ride. If we consider the patient, her supports and her medical professionals as one system, we can see that this system can fall under significant stress at just the time when calm, clarity and collaboration would best serve the patient's interests.

Medical and public health professionals agree that behavior change is crucial to prevent and manage chronic conditions (e.g., obesity, diabetes, arthritis) and improve the quality of treatment decisions for complex medical conditions (e.g., cancer, coronary arterial disease). But despite 50 years of academic research on behavior change and more than 15 years of academic research on medical decision making, sound evidence about how to accomplish these goals is sparse. To date, the cognitive paradigm in psychology has dominated the agenda of this research, omitting the pivotal roles of emotion and interpersonal dynamics in both patient behavior and treatment outcome (Livingstone & Gaffney, 2011).

Despite the dominance of the cognitive paradigm, the importance of emotion and relationships in patient care is well established. We know that emotional reactivity compromises intake of new information (Sanbonmatsu, Uchino, & Birmingham, 2011) and affects decision making (Lowenstein & Lerner, 2003). We also know that relational dynamics can trump intellect and information (LeBlanc,

Keeny, O'Connor, & Legare, 2009). The presence of an emotional connection with the health care provider is the gateway for patients, including the parents of young patients, to assimilate new medical information (Livingstone, Portnoi, Sherry, Rosenheim, & Onesti, 1969).

Yet the current medical system continues to separate the physical from the emotional and cognitive, focusing on physical symptoms with little time and attention given to how thoughts, beliefs and feelings impact bodily states and health behavior. The two national initiatives in patient-centered care, which were designed to lower costs, and remedy gaps in patient care (Institute of Medicine [IOM], 2001) and shared decision making (Braddock, 2010; Ottawa Decision Support Framework, 2006; Zikmund-Fisher et al., 2010), have not been widely implemented because they do not include ways for clinicians to effectively navigate the internal and interpersonal forces that impact their daily work. Medical care and the modality of health coaching would, in our view, improve with a paradigm shift, for which we offer one option.

This chapter illustrates a model of health and wellness coaching called self-aware informational nonjudgmental health coaching (SINHC™), an IFS-based coaching model that integrates the IFS approach of *direct access* with our new process *Information Interweave™*. We view health coaching as a modality rather than as a profession and consider SINHC an essential component skill. The concepts and clinical methods of the internal family systems model (IFS) have arrived at a fortuitous moment in the evolution of patient care. Many nurses have already learned components of SINHC, and we believe that a wide variety of health professionals could usefully add these skills to their education and training, for example, primary care physicians, physician assistants, physical therapists and medical social workers. Although the use of SINHC to provide “medical decision support” requires some knowledge about diseases and treatments, we believe that creative training programs, good informational databases and partnering with physician specialists will fulfill this requirement. Successful pilot studies indicate that additional research on health coaching is warranted (Appel et al., 2011; Wennberg, Marr, Lang, O’Malley, & Bennett, 2010; Wolever et al., 2010), and the specificity and clarity of SINHC methods should enable well-designed comparative studies to be conducted on its effectiveness.

Although direct access, one of the approaches sometimes used in IFS treatment, can sound like ordinary conversation to the outside observer, the SINHC-trained coach who is using it is actually listening for the feelings and beliefs of the patient’s personality part(s) (hereafter referred to simply as *part*). Throughout the SINHC process, while remaining within the patient’s comfort zone, the coach will engage the patient’s parts with interest and concern but will not offer alternative beliefs, feelings or advice. Her goal is to honor and validate the part(s) who then tends to soften and becomes willing to differentiate from the patient’s Self. This process, called *unblending*, helps other parts to relax and can often be observed in shifts of the patient’s tone and vocal intensity. The coach notes these shifts as well as the level of connection between her and the patient and then assesses the readiness of the patient for new information. As the patient appears ready, the coach communicates evidence-based information in a manner that connects to activated parts and matches the patient’s pace, literacy and cognitive style.

Our goal in creating SINHC was to offer health care professionals, including nurses and doctors, effective tools for meeting the needs of patients who must make behavioral changes to regain their health, or who face the emotional challenges inherent in making choices about their medical care. Health

professionals also have reactive parts and often feel frustrated when the treatment option viewed by the patient as most desirable yesterday is rejected today, or when an accepted surgical procedure evokes postoperative dissatisfaction and remorse. The daily stress of delivering care and interfacing with vulnerable patients can stir up feelings of helplessness and the urge to manage both the patient's choices and his feelings about illness. Yet contradictory behaviors in patients are not solved by additional information because they are not based on logic or rationality. They come from the feelings and beliefs of parts who are activated within the patient. In our experience, health professionals are often unaware of the emotions and behavioral tendencies aroused within them by patients. This is a blind spot that increases stress, compromises the professional's ability to empathize with the patient and blocks discernment of the patient's true needs. SINHC is specifically designed to provide health professionals with pathways to work with their own parts as well as with those of patients.

This chapter illustrates how the interviewing protocol of SINHC helps the coach as well as the patient discover and work with feelings, beliefs and action tendencies that affect health behavior change and decision making. To be clear, we use the terms *health coach*, *coach*, and *interviewer* interchangeably to refer to any health professional who learns and practices SINHC. The range of issues presented to health coaches is broad and includes any problem that appears to be associated with the patient's health issues. Here are some examples: a nursing home resident has been reported for resisting rehabilitation; the mother of a child has terminal cancer and is experiencing marital tension; a woman has had several miscarriages and is confused about whether to have amniocentesis; a man has quit taking his beta blockers because they make him feel depressed; a patient cannot decide which treatment (for breast cancer, prostate cancer, angina, etc.) is best for him or her. The SINHC protocol is not linear. It guides the interviewer to start by connecting with the patient's affect and exploring the concerns of the activated parts rather than focusing right away on medical facts, such as the importance of continuing on beta blockers. SINHC creates a road map between coach and patient, which helps them stay connected as the patient's needs are revealed. The interview may move well beyond medical concerns and parts to other relevant factors, including difficult economic and environmental circumstances and the insensitive behaviors of relatives or care providers.

SUMMARY OF THE SINHC PROCESS

Before starting an interview the coach becomes grounded in her body and notices her own parts (McConnell, this volume). Holding the intention to keep these parts relaxed and unintrusive, she maintains a double awareness of the patient's internal system and her own. Using this self-awareness during the interview, she checks frequently but briefly on any of her own parts who could interfere with her coaching goals, promising to listen to their concerns later. Her initial goal, in addition to feeling open and grounded, is to establish a connection with the patient's parts(s). Although protector parts often show up first, the patient may lead with vulnerable parts. The coach accepts all vulnerability with compassion but, in the initial contact, will refrain from stimulating any more emotion to keep vulnerable parts from blending, or overwhelming the patient.

The coach tracks the patient's internally generated conflicts as well as those activated by family or medical providers. To increase the patient's self-awareness, she expresses curiosity about the meaning of his emotional tone, sighs, pauses and any storyline contradictions. When opportunity arises, the coach encourages him to identify his feelings but may not call them "parts." Rather than providing him with answers, she stays affectively connected, follows clues and explores his parts' concerns. Emotional readiness for more information tends to emerge after parts have been acknowledged and met with understanding and compassion. Guided by the patient's readiness, the coach does the Information Interweave by offering small chunks of pertinent medical information.

Although training in SINHC includes learning some core concepts of IFS and some terminology, the coach refrains from teaching the IFS language to patients. Nevertheless, when the need arises and will serve the goals of self-awareness and unblending from extreme parts, some terms are used. For example, if the patient says he feels angry and scared about the diagnosis, the coach may respond, "I am hearing that one part of you is feeling angry and another part is feeling scared about the diagnosis. Does that sound accurate?" The interviewer stays close to the patient's language and feelings, affirming that it is normal to feel ambivalence and have conflicting agendas when making medical decisions and changing behavior (Table 9.1).

Table 9.1 SINHC™ Components

Internal strategies in coach (a through e) (Before starting and throughout)	Interpersonal strategies (1 through 11) (Leading priorities)
a. Connect with a sense of openness/relaxation in mind/body and hold intention to maintain this.	1. Make affective, open energetic contact with the patient.
b. Be aware of the agendas (goals/internal purposes) that might belong to your health care manager parts as different from the agendas of the patient's parts, without letting your activated parts be in the lead.	2. Build attuned connection with patient's presenting parts and find words for emotions.
c. Notice your internal reactions to the patient's specific illness and his attitudes and choices.	3. Begin tracking emotions, content and agendas of the different parts.
d. Focus on maintaining a double awareness with curiosity and compassion toward your internal system and that of the patient.	4. Establish clarity with the patient about the agenda of the meeting and integrate 1, 2, 3 of the above priorities until there is a sense of agreement between you and the patient about the focus of the contact.
e. Notice any internal reactions that signify which part of you has been activated by what the patient is saying about their relatives and care providers.	5. Create an opening for and acceptance of the patient's vulnerability; name it without expanding it or pulling for more.
	(Used in flexible order as indicated by the patient's process)

6. Interweave credible information guided by the receptivity of the patient.
7. Invite identification and exploration of beliefs, emotions, responsibilities of parts of the patient, particularly when they conflict or get activated during the interview.
8. Invite identification and some exploration of conflicting priorities generated by relationships with care providers and family.
9. Identify and explore factors supporting internal motivation for specific behavior changes and decision making.
10. Identify and explore cultural, economic and environmental influences.
11. Establish agreement with patient's parts on an action plan, on follow-up contacts, and referral to a therapist when appropriate.

CLINICAL EXAMPLES

The following three examples are an amalgam of frequent health predicaments that require behavior change and decision making. At key points in each health coaching interview below, we point out which components of SINHC are being used by referring to [Table 9.1](#) above.

Example 1: Behavior Change in Diabetes: “I'm Ready to Give Up”

The vulnerability inherent in life threatening chronic illness is striking. We have heard newly diagnosed diabetes patients express feelings of outrage, betrayal, shock, fear, grief or numbness. From the perspective of IFS, different parts hold these feelings. We have observed physicians and coaches reacting to these same patients with disconnected reassurance, inventories of scientific facts and lectures about complications. In our experience, diabetic patients who are referred for noncompliance provoke intense judgments and are labeled “resistant.” A health professional who is motivated by a part intent on teaching facts to diabetic patients will miss the opportunity to connect with the patient's parts. But it is exactly these parts in the patient who determine whether new information will be taken in and whether changes in health behaviors will occur. The diagnosis of diabetes 2 is life changing. Because it affects the entire body, with potentially life threatening complications involving the heart, blood vessels, eyes and kidneys, it requires many specific behavioral changes to lower health risks. In the beginning, the

need to track physical issues and institute changes may feel overwhelming for the patient. Areas of concern include four physical items that need to be followed: blood pressure, cholesterol, AIC (average three to four month blood glucose) and renal function; and four behavioral changes to be made: lose weight, increase physical activity, modify nutrition and quit smoking. The clinical vignette below illustrates the use of SINHC with a patient who had a recent onset of diabetes 2.

The patient, a 55-year-old widow speaking on the phone with her SINHC-trained coach, expressed frustration (a part) over wide oscillations in her blood sugar scores. Although her primary care doctor had assured her that wide oscillations in sugar level were normal at this point in managing her illness and had urged her to focus on monitoring blood pressure, changing diet and quitting cigarettes, she said to the coach, “My sugar scores are ridiculous compared to my sister’s. I’m ready to give up” (a part). Before responding, the coach checked her own internal state. She noticed tightness in her chest, which was making it hard to breathe fully and feel grounded in her body. By checking inside she realized that one of her own parts had tensed in response to the patient saying she was ready to give up (see [Table 9.1](#) : Internal strategy c). She was familiar with this part. It protected her from feeling like a failure, an issue from her childhood. The coach also had an older sister and was familiar with another one of her parts who felt competitive. She recognized that her parts had been activated by the part of the patient who was similar. She put her hand on her chest, allowing its pressure to open her breathing and settle her body. She acknowledged each of her parts, saying, “I’m here. I’ll check with you later. Right now I’m going to do my job.” She felt her chest open and her breathing begin to move again. Now she became fully curious about her patient’s experience (see [Table 9.1](#): Internal strategy e). First she helped the patient label her feelings, saying, “It sounds like you feel frustrated and discouraged—is that close?” The patient agreed. So the coach asked, “What is the most important thing about your scores being different from your sister’s scores?” (see [Table 9.1](#): Interpersonal 2, 3, 8).

The patient stopped to think before replying, “She’s always been ahead of me. We didn’t really get to be friends until we grew up. I’m afraid she’ll deal with her illness and I won’t.”

The coach checked inside to see how she was reacting to the patient now (see [Table 9.1](#): Internal b). She noticed curiosity and compassion. “Can you tell me more about this fear?” (see [Table 9.1](#): Interpersonal strategy 5).

“My grandmother had to have her leg amputated because of diabetes. It was awful,” the patient said.

“So does it make sense for you to feel scared?” the coach asked (see [Table 9.1](#): Interpersonal 5). “Yes, my sister was told that if she didn’t get her blood sugar under control, her circulation would be affected and she could lose both her legs.”

“It’s really important for us to be aware of all your fears,” the coach said. “And when we talk about something scary, we can slow down to notice what’s happening in your body because that’s where feelings start.”

“It’s kind of hard to breathe when I think about having my legs cut off,” the patient said.

“Would it be OK just to breathe calmly together for a minute, then?” (see [Table 9.1](#): Interpersonal 5).

“Yes, I’ll try it.”

“OK. Let’s slow down. Notice that you are inhaling and then slowly turn it around and exhale. Now take a deep breath letting it go all the way inside. Then let it out slowly. How’re you doing?”

“Better. My heart is slower. I feel calmer.”

Sensing that the patient might now be ready for Information Interweave, the coach asked, “Would you be open to hearing about research on circulation and blood sugar? You may have already heard it and if you forgot that’s OK”

With the patient’s consent, the coach explained, in user-friendly terms, research indicating that other issues such as blood pressure and blood cholesterol are more associated with peripheral vascular disease than daily fluctuations in blood sugar. She added that fluctuations in numbers for sugar level were common in the beginning of regulating blood sugar. Then she said, “To be sure I’m clear, would you let me know if what I just said made sense?” (see [Table 9.1: Interpersonal 6](#)). The patient sighed with relief and repeated the pertinent research findings.

The coach, aware that relational dynamics often trump credible information, then asked, “What is it like to hear that your sister didn’t understand all this information?” (see [Table 9.1: Interpersonal 8](#)).

“I feel irritated,” the patient replied. “And I guess surprised too. She always sounds right.”

“Irritated and surprised,” the coach repeated. “What is it like to pay attention to these feelings?”

“Oh it helps me slow down. I see I’m just beginning to learn about diabetes and I know my sister wants to help. There’s so much for both of us to learn—it can be overwhelming.”

“Would written information about complications be helpful?” the coach asked (see [Table 9.1: Interpersonal 6](#)).

“I’m open to learning more,” the patient replied.

The coach did not tell the patient to stop smoking or change her diet at this point, however. She offered to send her written information and a DVD that they could go over another time. Because the interview was almost over and the coach knew that acknowledging extreme statements lowers the risk of maladaptive behavior, she returned to the patient’s opening statement.

“When we began talking, I heard you say you felt like giving up. How are you feeling now?”

“Oh I’ll keep checking my blood sugar numbers every day. And I’m going to see my doctor next week. I’ll ask her about complications too.”

The coach and patient agreed to be in contact as she visited her doctor. They also agreed to focus one by one on the eight items that are key in controlling diabetes over time: four to monitor and four to change. Finally the coach asked, “Would you be interested in joining a group to support your learning?” (see [Table 9.1: Interpersonal 11](#)).

“If I could find one near home and maybe one that my sister could come to as well, I think I’d like to try it.”

In a 55-minute encounter this nurse health coach helped the patient process strong emotions and integrate important medical information. In the future, if the patient’s busy providers or the group she joined were to disappoint her, she would have this experience as a reminder to call up, not give up.

Example 2: Decision Crisis in Coronary Disease: “Just as Soon Be Done With It”

People with coronary arterial disease (CAD) and the chest pain of angina face possible sudden heart attack and death. Treatment decisions are urgent and complex. It is common for patients to feel

overwhelmed by the information they need to assimilate while feeling extremely vulnerable. Culture and custom, family and career factors all affect options and behavior change. A health coach can hold the time and space a patient needs to process emotions and beliefs, then hear and sort out evidence-based information to make a “good enough” decision.

This 59-year-old schoolteacher was the father of two school-age children. He lived in a rural community, was in a second marriage and had debilitating angina. He initiated the first encounter with the health coach, a former intensive care unit (ICU) nurse, saying he “only” wanted more information about angina (presenting part). He said he had spoken with his primary care doctor; what he heard scared him and he had already scheduled a consultation with a cardiologist. They scheduled a follow-up call. He started this second telephone coaching encounter, which followed his first session with the cardiologist who had supplied him with abundant information on his condition and treatment options, by saying, “I’m not enjoying my life very much; I’d just as soon be done with it.”

The coach responded, “You’re not enjoying your life. What is it like to feel that you could be done with it?” (see [Table 9.1: Interpersonal 2](#)). At the same time the coach noticed her own body stiffening and identified a managerial part who had the agenda of saving his life. She went inside for about seven seconds and asked this part to soften so she could stay with the patient’s unhappiness and hopelessness (see [Table 9.1: Internal c, d](#)). Knowing that the patient’s opening statement came from one part, she wanted to hear more of his story.

But first she helped him find words that fit his emotions, “It sounds like the words ‘discouraged’ and ‘hopeless’ might fit what you’re feeling about having so much information and having angina pain—what is your sense?” She did not try to talk him out of this state with facts about successful treatment nor did she ask about past experiences of surmounting discouragement. Instead she held space and time for his feelings without rushing him (see [Table 9.1: Interpersonal 5](#)).

The patient then shifted and began to describe how talking to the cardiologist about treatment options had left him feeling confused and hopeless. “When he started listing the survival rates, I felt completely overwhelmed and realized I would never be able to sort it all out,” he reported.

“Is that what you meant when you said you feel ‘done with it’?” she asked. When he confirmed, she went on, “What words would best describe your experience with the cardiologist?” (see [Table 9.1: Interpersonal 5](#)). He thought for a moment before answering, “Confused—disappointed—annoyed—hurried.”

“You know I’ve talked with a lot of folks in your position and that response is not uncommon. All of that information and pressure to decide can be overwhelming.”

He let out a sigh and said, “It was daunting.”

She repeated, “Daunting.” Then to explore and track his parts, she asked, “What was the hardest thing about your conversation with the cardiologist?”

He replied, “It was the pressure to make a decision quickly because I could die. And then he can’t guarantee relief from the angina anyway. I wanted to make a decision but I just couldn’t.”

She asked, “What’s the most important thing about your angina?” (see [Table 9.1: Interpersonal 5, 7](#)).

“It’s about my daily life. My boys love soccer and want me to play with them and my wife likes having sex—so do I. Right now I’m failing them all.”

The coach helped him find a feeling word for this thought. It was “sad.” She acknowledged the emotion and also the importance for him of being active with his sons and wife. She held the space for him to honor what was important and then asked whether “more information about relief from angina might be useful at this point” (see [Table 9.1: Interpersonal 6](#)).

“Yes,” he replied.

She knew that the cardiologist would have given him most of the information already, and she heard that he was stuck on the disclaimer about no guarantee of relief from angina. She explained slowly that all possible choices (meds alone, bypass, angioplasty, bypass surgery) offered some angina relief, and bypass offered the most. She had looked this up ahead of time so she would not disrupt their connection by reading it to him from her database. As he heard the information, he felt surprised, which moved to his next concern: that he would die during bypass surgery. “One day that’s my biggest fear,” he said. “The next day it’s about a stroke and failing my family. Then the next day I’m terrified if I don’t do something right now I’ll die from a massive heart attack on the soccer field or while having sex.”

“What is all this like for you?” she asked.

“Crazy making!” he said. “How can I decide? My wife wants to go the angioplasty route because she’s afraid of heart surgery. But if I don’t get angina relief with angioplasty, I’ll have to do the bypass someday anyway. It’s too much!”

“It makes sense that you have many different feelings and responses. Your wife has strong feelings too. All of this may not feel good, but you are in a normal process of making the decision that will be good enough for you and all your emotions and priorities; it’s a process that needs enough time and consideration. But it usually saves time to take the time” (see [Table 9.1: Interpersonal 7](#)). “Let’s go over the story so far. Tell me if this seems right. For you, an active life is very important. You also take responsibilities seriously. You don’t want to die and leave your children without a father and your wife without a partner.”

“No I don’t. That’s what happened to me. My father died when I was eight.”

“So you are recalling that experience? Anything you want to share right now about it?”

“It was like a hole in my heart,” he said.

She acknowledged the intensity of his feeling. “Does it make sense this decision would be extra hard for you?” she asked (see [Table 9.1: Interpersonal 5](#)).

“Yes,” he said.

As important as traumatized parts are, the adult who needs to make a decision in the here and now is at a disadvantage when blended with vulnerable young parts, so the goal of the coach was to stay with the present. At the same time she was aware that he might not be able to unblend from his vulnerability right now. So while they talked, she was evaluating the need to make an immediate referral for therapy. If he were able to unblend enough now, she would remember to offer him information about IFS therapy at a later time (see [Table 9.1: Interpersonal 11](#)). To see if he could stay in the present, she asked, “Would it be OK to ask your feelings about your father’s death to step back right now, letting them know that you are not forgetting them and setting the intention to come back when you have time?”

When he agreed, she returned to a respectful review of the conflicts between his parts about his treatment choices—a process of “shared decision making” guided by SINHC. As they neared the end of the appointment, she offered, “We can speak again soon to summarize all this. It usually helps to go over

each option and assess how you feel one step at a time with just as much medical information as you may need for each step” (see [Table 9.1: Interpersonal 6](#)).

“Here’s what I want to know,” he said. “How can I reduce my chances of dying during bypass surgery? And what is the best choice to decrease angina? Then I need to talk with my wife.”

“OK, I’m hearing that you are clear about what you want to know,” she replied. “Shall we talk again this week to develop questions for the cardiologist and your primary care doctor so you can get this information at a pace that is reasonable for you?” (see [Table 9.1: Interpersonal 9](#)).

They followed this phone call with four more sessions, each about 30 minutes, over a period of three weeks. The patient returned to his doctors with specific questions to slow the process down and to assimilate what research showed: a 98 percent survival rate for bypass teams that were doing over 400 cases per year, with a 95 percent chance of not having a heart attack and a 99 percent chance of not having a stroke from the procedure. Angina relief was 90 percent five years out from bypass but only 80 percent with angioplasty. At the same time, bypass has a 10 percent to 30 percent chance of causing cognitive impairment that could persist for five years. Having learned that all CAD patients need medication in any case, the patient also needed to process his reaction to the potential side effects of over eight medications, for example, statins, angiotensin-converting enzyme inhibitors, calcium channel blockers and so forth. He processed many pieces of this kind of unnerving information with the coach and some with his doctors.

In a joint telephone call, the coach helped both the patient and his wife to start speaking for their fears rather than to act them out (see [Table 9.1: Interpersonal 8](#)). His wife shared that her uncle had died on the table during surgery for a heart condition. After the coach interwove some relevant statistical information, the wife stated, “That sounds much better than I expected. My uncle’s surgery was a long time ago.” She was encouraged to continue honoring her fear verbally, and the patient was encouraged to listen with compassion but without blending with her fearful part or with his part who wanted to rescue her from feeling fear.

Once the patient had listened to his wife’s fears as well as his own feelings and beliefs about his options and risks, the decision could unfold as the product of a holistic process. He said, “I guess I’ve heard from most of the feelings of my parts and I understand the information.”

He chose coronary bypass at the city hospital. However it might turn out, he had a strong sense that he had considered all the essential information and had done his best to be prepared emotionally and relationally. This shared, “good enough” decision was a product of the SINHC coaching process.

Example 3: A Decision About Having Cervical Spine-Fusion

Surgery: “Upset and Confused”

A patient who is in a state of vulnerability from pain and physical limitations is likely to be very frightened when shown structural malformations in a spinal X-ray and an MRI. Under such circumstances the patient will tend to believe the doctor’s interpretation of the malformations along with whatever prognosis is offered, even if the doctor does not understand how the patient’s lifestyle and history could affect the prognosis and is therefore not well prepared to orchestrate shared decision

making. Lack of communication about the patient's needs may make it hard, in turn, for the patient to give consent for treatment that is truly well informed.

In this example, a 65-year-old woman thought she needed to make an urgent decision about cervical spinal fusion surgery. She had spinal congenital stenosis with ankylosing spondylitis and bulging discs that extended from her lumbar to her neck region, for which a metal rod that stabilized her spine from her lumbar to thoracic region had been successfully inserted. Although she had a good relationship with her surgical team, a neurosurgeon and an orthopedic surgeon, she had moved to a different city and, feeling worried about numbness, tingling and weakness in both arms, had made an appointment for evaluation with an orthopedic surgeon in her new city.

After reviewing her MRI of two days prior, this new surgeon had recommended immediate cervical surgery to fuse destabilized discs. If she did not do this, he said she would run the risk of irreversible damage. Moreover, she should not drive without a stiff neck support because if she were rear-ended she could be paralyzed. As she left this appointment, she felt upset and confused. She called the health coach and shared her story. The coach validated her presenting dilemma. "I hear the urgency in your voice. It sounds like one part of you is feeling really anxious and is hearing the immediate need for surgery, and another part is confused about this procedure and needs more information about recovery, especially since you live alone with two pets. Is that right?" (see [Table 9.1](#): Interpersonal 2, 7).

"Yes," the patient replied.

"Would it be OK if we slow down and acknowledge these different feelings to see what more they might need you to know?" (see [Table 9.1](#): Interpersonal 3).

"Yes, I'm so upset about the speed of this decision. I don't even know how to plan for it. The surgeon was like a cowboy—he walked in shooting facts from the hip. I had no time to ask questions. Now I need answers but feel I don't have time to get them because he said I have to do this right now."

The patient was well informed about her medical condition and was as actively invested in a healthy recovery as she had been with her former surgery. When the coach asked her how she felt after this experience, she said, "Terrible."

"Do you understand this doctor's diagnosis and recommendations?"

"Well, he's the big expert in reading MRIs. He has a reputation for seeing things that nobody else sees. And, he says that I need surgery now."

"And what is that like for you?" (see [Table 9.1](#): Interpersonal 5).

"He was extremely self-confident and I didn't feel any connection with him. He didn't ask questions about me, or what I might need for recovery; he just told me what I should do and left. Frankly, he scared me. I don't want this surgery but if I need it, at least I want to know what to expect so I can take care of myself afterward. That's also scary. I did get a stiff neck brace for driving."

"That makes very good sense," the coach said. "Would you feel OK about calling the doctor's office to get your questions about surgery and recovery answered?"

The patient said she would do so the next day. In their follow-up call she reported that she did not get her questions answered by the surgeon's staff, instead she felt pressured to schedule the surgery, which she was not ready to do. She felt more anxious.

"What did you discuss in your previous surgery?" the health coach asked (see [Table 9.1](#): Interpersonal, 9).

“It was completely different. You know, I actually travelled to this hospital three years ago to see this surgeon and left without scheduling surgery for another year because he scared me then, too. I saw a bunch of other surgeons at different places and finally scheduled with the ones who seemed concerned about me and explained everything, and then a nurse even called to ask if I had other questions. She was willing to walk me through the surgery and the recovery in detail. They were warm and also confident—and they listened!”

The coach had to work within herself to relax a part who wanted to help the patient (see [Table 9.1: Internal a–e](#)). “May I ask how you decided to return to the surgeon here?” she said (see [Table 9.1: Interpersonal 8](#)).

“I moved and so I live closer to him now. It’s supposed to be the best teaching hospital and he’s supposed to be the best at reading MRIs.”

“And how important does reputation feel to you right now?” the coach asked (see [Table 9.1: Interpersonal 7](#)).

“I guess it’s not enough to make me feel confident when he behaves that way. I’m scared and I can’t bring myself to schedule surgery with him.”

“What do you need to feel as confident now about your treatment as you felt with your previous surgeon?” (see [Table 9.1: Interpersonal 7](#)).

“I need surgeons who are willing to answer questions and collaborate. I wish I could speak to my other team about this surgery.”

“Can you send your MRI to your former team and have this discussion with them?” (see [Table 9.1: Interpersonal 9](#)).

“Wow!” said the patient. “That’s exactly what I need to do. I feel better already.” Later she reported to the coach that she had an appointment with her former surgeon who had taken a careful history and discovered the following: she had been lifting and bending due to her recent move in ways that aggravated her spinal condition, but because of her move she had resumed physical therapy and the numbness, tingling and weakness in her arms was actually abating. Although he saw the same condition the other surgeon had seen on the MRI, he observed that things were moving in the right direction. Together they reviewed her symptoms and shared the decision that she should schedule surgery immediately if the symptoms got worse, but if she continued to improve she could take some time. This seemed right to her and she now had time to create a post-op recovery plan should she need surgery. At one-year and two-year follow-ups, the patient had continued to improve and did not require further surgical intervention.

As we have seen in the three examples above, coaches are trained to make and maintain affective contact with the patient with a friendly tone and willingness to listen. The coach acknowledges the patient’s presenting emotional and cognitive states (parts), leaves space for emerging vulnerability rather than breaking relational contact by switching to an informational mode, facilitates the patient in being aware of internal reactions to medical providers and family members, and acknowledges all emotions and beliefs, including contradictory ones, that the patient experiences about making behavior changes and decisions. In addition, the coach helps the patient to identify cultural and economic influences and to clarify what he wants and needs from health providers now and in the future.

CONCLUSIONS

The key to SINHC coaching lies in a compassionate listener who is clear about her goals guiding activated parts of the patient to unblend. In our experience, unblending decreases unwarranted treatment choices, unstable decisions, postdecision remorse, discontinuation of medication, behavioral paralysis and relational conflict between patients, their providers and families. Internal disagreement between parts inside the patient or between a part of the patient and a part of the health provider is normal yet is often mis-interpreted as noncompliance. Repeated polarizations of this kind between healthcare professionals and patients over normal behavior are a loss to health programs for all involved. By decreasing misunderstandings and miscommunications and enhancing information transfer, SINHC can help to increase economic efficiency in health care.

Our health care system has made nationwide efforts to improve the quality of medical decision making as well as the prevention and management of chronic conditions. Despite improvements in evidence-based decision aids ([Informed Medical Decision Making](#), n.d.), these programs continue to be hampered by lack of a valid, comprehensive model that takes account of the emotional, physical, cognitive, intra- and interpersonal experience set in motion by illness and efforts to change health behavior. Going beyond the recent emphasis on relationships and emotional states in behavior change (Donadio, 2011), SINHC is a comprehensive model of health behavior change and decision making for health coaching, evidence-based medicine and patient-centered care that integrates the direct access approach of IFS with Information Interweave and synchronizes the power of attending to emotions, beliefs and bodily sensations with the power of knowledge.

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